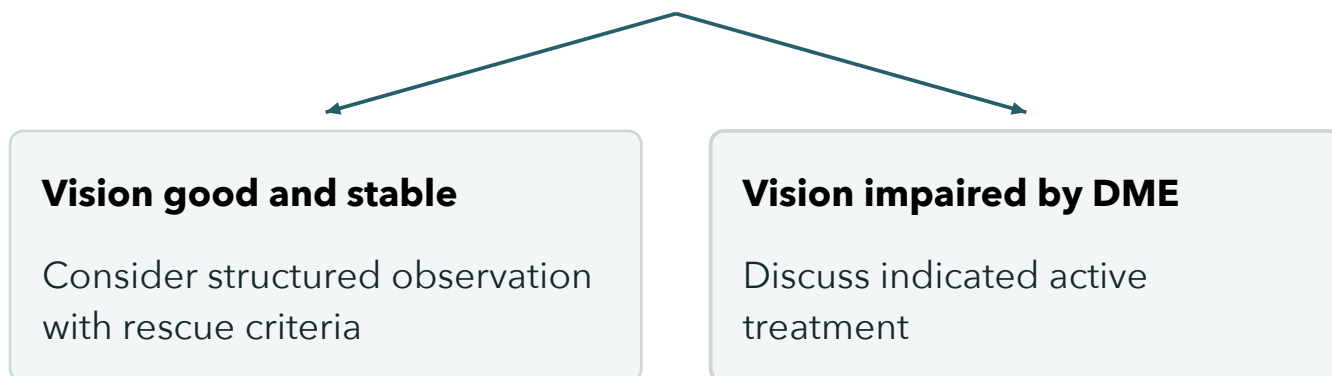


DME good vision persistent fluid and drug selection

Part 2 | Good-vision DME: observation is active care

A good acuity changes the initial treatment discussion.

Centre-involving DME with reliable review



Protocol V concerns a selected good-vision population. Observation means planned visits, repeatable acuity testing and prompt access to rescue. It is not a reason to ignore OCT, systemic risk or a patient who cannot reliably return.

Protocol T: read the subgroup and time point

Comparative efficacy depends on baseline vision.

Finding	Implication
Better baseline vision	Similar average visual gains among the three agents
Poorer baseline vision at one year	Aflibercept advantage over both comparators
Poorer baseline vision at two years	Advantage over bevacizumab, not ranibizumab
Clinical choice	Include access, safety and treatment burden

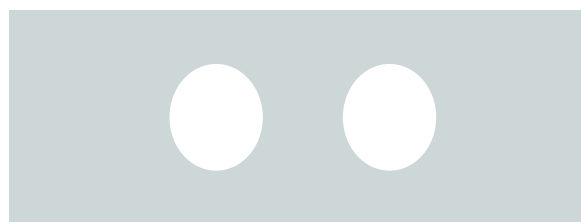
A headline of superiority is incomplete without its population and endpoint. Avoid promising the average trial gain to an individual or assuming a result between two agents establishes superiority over newer treatments not tested in the trial.

Persistent fluid: assess the whole response

An early incomplete response does not establish permanent failure.

Schematic OCT sections. A dark space is not automatically treatable oedema.

DME: thickened



Oedematous fluid

MacTel: thin cavities



Degenerative cavitation

Original OCT contrast: thickened oedema versus thin degenerative cavitation.

Some eyes with a small early vision gain improve later without switching. Central thickness alone is an imperfect predictor of later acuity. State what has improved, what remains abnormal and whether a different mechanism explains the discrepancy before declaring anti-VEGF failure.

Steroid implant: identify the eye-specific cost

Pseudophakia reduces one concern but does not remove all risk.

Finding	Implication
Potential advantage	Alternative sustained local anti-inflammatory delivery
Phakic lens	Cataract progression risk
Pressure vulnerability	Assess and monitor steroid-related IOP elevation
Active infection	Reconsider suitability before local immunosuppression

The UK Ozurdex indication and contraindications should be read for the actual eye. A lower visit burden is not a substitute for monitoring, and a previous steroid pressure response may materially change the balance.

Migration is a structural complication

Capsular and zonular anatomy can expose the cornea to the implant.

- Read the original cataract and vitrectomy records
- ↓
- Assess capsule, zonules and IOL support
- ↓
- Identify a route to the anterior chamber
- ↓
- Choose a safer plan if migration risk is unacceptable

A new corneal problem after implantation requires localisation of the implant, not an assumption of routine post-injection irritation. Migration management is urgent when the endothelium is threatened; involve the vitreoretinal team and consider the continuing risk of remigration.